Trifluridine/tipiracil (Lonsurf) monotherapy

Treatment scope · NCCP 00382 This guide covers Lonsurf on its own for previously treated metastatic colorectal cancer under NCCP 00382a. It does not cover gastric cancer or Lonsurf with bevacizumab. Confirm the exact regimen on your prescription.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cyclic oral monotherapy: moderate-to-high emetic risk

Active medicines: trifluridine_tipiracil

Emetogenic risk: moderate_to_high

Medicine	Dose / route	When
One oral 5-HT3 antagonist (choose one)	National Cancer Control Programme (NCCP) oral reference options: granisetron 1–2 mg total daily OR ondansetron 8–16 mg total daily	Start before oral treatment and continue daily according to the prescribed course; team specifies dosing and rest-day coverage Alternatives, not concurrent prescriptions. Assess QT risk, constipation, interacting medicines, organ function and local formulary. No routine added steroid is inferred from parenteral guidance.

Additional phase-specific support

- Agree oral preventive antiemetic and rescue instructions under NCCP oral guidance/local policy; this is not the NCIS parenteral moderate/high protocol.
- No routine premedication is specified by NCCP 00382.
- Assess need for granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or other infection support individually; no universal prophylaxis is prescribed here.
- Use the exact dosing calendar and no catch-up doses.
- Reconcile gastrointestinal (GI) symptoms, hydration and additional blood counts for renal impairment.
- Confirm the body-surface-area prescription, actual morning and evening tablet counts and Days 1–5 and 8–12 calendar. Take within one hour after breakfast and the evening meal; do not replace missed or vomited doses.
- Follow current NCCP cycle-start blood and urine checks and the additional EMA blood-count requirement before starting and as needed, at minimum before each cycle; increase monitoring with renal impairment or toxicity.
- Prior reductions must be retained: no dose re-escalation. The severe-renal dosing pathway differs from the standard minimum dose and needs separate review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00382 v4 · p1–6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required